

Enhance the GLP-1 Weight-Management Enrollment Flow — Older-Adult CVR Audit

Conversion audit of the Figma enrollment flow (node 415:14957) for the GLP-1 Weight-Management funnel, 65+ Medicare. Grounded in the Bold Funnel Conversion Audit, bullseye research, and senior-onboarding evidence.

Context

Goal. Raise conversion (start -> scheduled visit, and scheduled -> show) for the GLP-1 weight-management enrollment funnel, viewed through the eyes of a Medicare-age adult (our bullseye persona: e.g. *Jennifer — tried semaglutide, paid \$400/mo, quit from nausea, now wants a pill and a doctor who keeps her on track*).

What's being audited. The Figma board *Weight management* (Mc7PfQwn4QNIXM6HKOTT2v, node 415:14957) — treated as the source of truth per the user. Where the built `PrevMed/glp1_funnel/*.html` prototype already solves a problem, it's flagged as **"already built — port it into Figma"** rather than re-specified.

Evidence base (every recommendation cites at least one):

- *Bold Funnel Conversion Audit* (senior/Medicare persona, step-by-step drop-off + Tier 1–4 stack).
- *Bullseye research* — [findings.md](#), [principles.md](#), [value-props.md](#), [positioning.md](#), [glp1-messaging.md](#), [clinical-protocol.md](#).
- *Senior-onboarding playbook + conversational-vs-page-by-page* (page-by-page is correct for clinical intake; chunk, don't chat).

The one strategic frame (from the audit): *every screen must answer — does this 72-year-old know what they just agreed to, why, and what happens next?* The funnel currently escalates the ask (PII -> third-party consent -> clinical exclusions -> 45-min Zoom) faster than it builds *trust*. Optimize for **commitment, not just completion** — a senior who finishes booking without internalizing it is the profile that no-shows.

The Figma flow as it stands (node IDs)

1. **Landing** 415:8506 — "Doctor-supervised weight loss, Covered by Medicare" + "\$50 monthly copay"; First/Last/Email; CTA **"Check coverage"**; carrier trust line; older-adult video image; star rating.
2. **Eligibility** 415:8932 — "Help us confirm your coverage"; State dropdown; DOB (01/01/19|YY); **"Verified (Bold's service provider) and its vendors..." consent (gray)**; "Powered by Verified"; Continue.
3. **Intro interstitial** 415:8756 -> **Appointments covered** 415:9049 — UHC \$0 copay; insurance card.
4. **Intake Q1 motivation** 415:9255 -> **Health conditions (14-checkbox)** 415:9197 (Option 1 categorize vs Option 2 single-page stickies) -> **Height/Weight** 416:19951 -> **Med history** 412:5476 -> **Comorbidities (8-checkbox "Complete exclusion")** 412:4969 -> **Transition** 415:9241.
5. **Clinical result** appropriate 415:9327 / not-candidate 415:9328 -> **Intake Q3 goals** 415:19144/19201 -> **Match loader** 415:9795.
6. **Match result** candidate 420:11763/11803 ("\$0-25 out of pocket" + "\$50 monthly copay"; "We submit to Medicare to get your GLP-1 covered") / lifestyle 420:12081/12124.
7. **Schedule** intake-call 415:10688 / provider 415:10762 ("...help with {weight management.}"; 45-min Zoom; **Video default**; PT-only slots; **"Schedule appointment"**).
8. **Confirmation** 415:18911 / 415:14676 / 415:17629 — "You are all set!"; Zoom link; coverage ("\$50 month copay").

Each item below is tagged **[EXP]** (experience) / **[COPY]** (copy clarity), with: *what to improve -> why current fails -> rationale -> rewrite*.

TIER 1 — Biggest CVR / show-rate levers (do these first)

1.1 Landing: lead with the validated differentiator; keep the coverage hook but tell them what's next · [EXP+COPY] 415:8506

- **What:** Change H1 to the research-winning phrase; add a secondary **"See how it works"** scroll CTA and a **visible phone number**; keep the **"Check my coverage"** CTA but add one line of expectation under it.
- **Why current fails:** Hero leads with "Doctor-supervised weight loss" — generic. The form takes the fold with no signal to scroll (audit: ~70% don't scroll), no phone (seniors trust phone), and "Check coverage" alone reads as an insurance-lookup, not "start a \$0 doctor visit."
- **Rationale:** "Lose fat, not strength" / "strength, not restriction" was the single most resonant phrase, 4/4 bullseye (findings #17, #25; principles #13). "Check My Coverage" is a validated trust anchor + conversion trigger (findings #26) — so **keep the phrase**, fix the missing "what happens next." Audit Step 0: add provider faces, "\$0" proof, and a learn-more scroll path.

Note the tension: the generic audit calls "Check coverage" vague jargon, but our own research validates it. Resolution: keep the words, remove the ambiguity around them.

- **Rewrite:**
 - H1: **"Lose weight. Stay strong. Covered by Medicare."**
 - Sub: "Doctor-supervised weight care for adults 65+ — with FDA-approved GLP-1 medications for a \$50 monthly copay if they're right for you."
 - Primary CTA: **"Check my coverage"** -> microline beneath: *"Takes 1 minute. Most patients pay \$0 for their visit."*
 - Secondary CTA: **"See how it works"** (scrolls).
 - Persistent header/footer: **"Questions? Call (424) 577-5266."**

1.2 Eligibility: kill the "Verified and its vendors" consent; plain-English privacy; explain DOB inline · [EXP+COPY] 415:8932

- **What:** Remove the "Verified (Bold's service provider) and its vendors" language and "Powered by Verified." Replace with a short plain-English privacy line + lock icon. Put **"why we ask"** inline next to DOB, and fix the 01/01/19|YY placeholder.
- **Why current fails:** To a 76-year-old who reads AARP scam warnings, an unexplained third party ("Verified ... and its vendors ... may receive your personal info") right after the ad pattern-matches to identity theft. The text is also low-contrast gray, and the screen is mostly empty above a wall of legal copy — the scariest possible first impression. The DOB placeholder 01/01/19|YY is confusing and looks pre-filled/broken.
- **Rationale:** Audit Tier 4 names this exact "Verified and its vendors" pattern as a trust blocker; senior playbook Principle 4 (earn trust, no unexplained third parties) + the "blame themselves 90%" finding mean this step bleeds users silently. Inline "why we ask" beats a click (seniors avoid extra clicks). Gray-on-white fails the contrast floor (Principle 1).
- **Rewrite:**
 - Privacy line (dark, readable): **"Your information is private and protected (HIPAA). We use a secure service to check your Medicare coverage instantly — only your Bold care team ever sees your health details."** + plain "What we share" link.
 - DOB field: label **"Date of birth"**, helper inline: *"This confirms you're eligible for Medicare-covered care."*, format hint MM / DD / YYYY (not a fake-filled placeholder).
 - Button: **"Check my coverage"** (the action), not "Continue"/"Agree."

1.3 Make "you're covered" the emotional peak — personalized \$0, not a "\$0–\$25" range · [EXP+COPY] 415:9049, 420:11763/11803

- **What:** Replace every "\$0-25 out of pocket" range with a **resolved, personalized number** ("Your visits are \$0") and pair it with the **"78% of Bold patients pay \$0"** proof. Anchor the \$50/mo GLP-1 copay against the price they fear.
- **Why current fails:** A fixed-income senior reading "\$0–\$25" stops on *"which one is me?"* — and the audit shows this ambiguity sends them to "call Medicare first," never to return. "\$50 monthly copay" floats with no anchor, so it reads as a cost, not a deal.
- **Rationale:** Cost is the #1 historical blocker for this audience ([findings #20](#)); \$50 vs the \$300–700 anchor scored 9/10 willingness ([findings #19](#)); "78% pay \$0" is the standout proof to pair with any coverage claim ([findings #21](#), [principles #11](#)). Audit Tier 2: resolve cost ambiguity at the covered screen.
- **Rewrite:**
 - Covered screen: **"Good news — your visits are covered. Most Bold patients pay \$0."**
 - Result cost card: **"\$0 — most Bold visits, covered by Medicare." + "\$50/month — FDA-approved GLP-1, if it's right for you (vs. \$300–\$700 on your own)."**
 - Retire "\$0-25" everywhere; if a true range exists, show the personalized resolved figure.

1.4 Add a warm intro + reassurance *before* clinical screening; soften the heavy screens · [EXP+COPY] (new screen; [415:9197](#), [412:4969](#))

- **What:** Insert the prototype's warm intro before the condition/exclusion screens, and carry its reassurance onto each clinical screen. Port from [glp1_funnel/01-onboarding.html](#) (intro + safety framing).
- **Why current fails:** The Figma flow jumps from "you're covered" straight into clinical exclusions (kidney disease, cancer treatment, eating disorder). The audit calls this the funnel's **"biggest emotional landmine"** — the tone-shift makes a senior feel they walked into a serious clinic ("Am I sick enough? Am I too sick?") and depresses willingness to continue. There's no "no wrong answers" framing in Figma.
- **Rationale:** Audit Tier 2 ("A few quick safety questions so your doctor can plan your visit"); Principle 3 (warmth, never clinical/sterile) + Principle 5 (never make them feel at fault); brand-coherence (the ad sold "feel better," the funnel can't read as an ER intake). Non-shaming tone is a *clinical* principle here, not a nicety ([findings #50](#)).
- **Rewrite (intro):** **"Let's set up your plan.** A few quick questions help your Bold provider plan a safe, personalized approach — including an FDA-approved GLP-1 if it's right for you. About two minutes." + two reassurances: **" Private — only your Bold care team sees this." / " No answer rules you out — we'll support your health either way."**
- **Rewrite (each clinical screen subhead):** "These help your provider keep you safe. Checking a box never rules you out — your provider just reviews it with you."

1.5 Schedule: make phone equal to (or default for 75+) video; reframe "45-min Zoom"; local time · [EXP+COPY] [415:10762](#)

- **What:** Present **Phone** and **Video** as equal choices (default **Phone** for 75+); reframe the 45-min Zoom commitment; show times in the **user's local timezone**.
- **Why current fails:** "Video Call" is pre-selected (purple) and the screen leads with **"45-min Zoom video call"** — a huge first-touch commitment that seniors book out of politeness then no-show because they can't get Zoom working. **PT-only** slots ("8:00am PT") confuse East-Coast users. This screen is the single most no-show-prone moment.
- **Rationale:** Audit Tier 1 — "make phone visit equal to Zoom (two buttons, not a hidden link); default to phone for 75+"; seniors no-show Zoom at 2–4× phone; reframe duration as "15-min intro + plan"; show local timezone. AARP: 66% without telehealth prefer audio-only.
- **Rewrite:**
 - H1: **"Choose a time for your first visit."**
 - Sub: **"Your first visit is a 45-minute call with Dr. Desai — by phone or video, your choice."** (toggle defaults to **Phone** for 75+).

- Slots: render in the member's local time (e.g. "11:00 AM ET"), not PT.
- Optional reframe: "We'll start with a short welcome, then build your plan together."

TIER 2 — Funnel hygiene & cognitive load (reduce mid-funnel drop)

2.1 Add "Step X of Y" + persistent help phone on every step · [EXP] (port from HTML)

- **What:** Replace the thin unlabeled progress bar with a labeled "Step 3 of 6" and keep "Need help? Call (424) 577-5266" visible on every step. **Already built in `glp1_funnel` — port to Figma.**
- **Why current fails:** Figma shows only a hairline bar; the user can't tell if they're on step 2 of 3 or 2 of 12 (audit Step 1 drop-off reason #4). No phone is visible until after commitment.
- **Rationale:** Senior playbook — visible progress reduces anxiety (Transparency); persistent phone is the highest-ROI support feature and seniors prefer phone for resolution. Conversational-vs-page-by-page gates #3–4.

2.2 Cut and consolidate the question set; group the 14 conditions; replace open-text with chips · [EXP+COPY] 415:9255, 415:9197, 412:4969, 415:19144

- **What:** (a) **Merge** the two condition checklists — the 14-item "Health conditions" 415:9197 and the 8-item "Complete exclusion" 412:4969 overlap and read as being screened twice. (b) **Drop or merge** the redundant *Intake Q1 "motivation"* 415:9255 and *Intake Q3 "what to talk about"* 415:19144 — they ask nearly the same thing. (c) For the conditions screen, ship **Option 1 (categorized groups)**, not Option 2. (d) Make any open-text **optional with chips + voice + a real Skip**.
- **Why current fails:** More fields = lower CVR (the 5-to-7 field cliff; every field past 5 ~ –2.8 pts). Two near-identical exclusion screens and two near-identical "goals" screens inflate perceived length and invite "didn't I already answer this?" Open-text on mobile is the #1 abandonment pattern for 65+.
- **Rationale:** Audit Tier 2 ("replace open-text with chip-select + one optional textarea"); field-count cliff (senior playbook). For the 14 conditions specifically, **categorized grouping (Option 1)** is the right call — recognition-over-recall with all options visible beats both an ungrouped wall and splitting across pages (NN/g chunking; matches the prior grouping recommendation: *Diabetes & related · Mental health & wellbeing · Other health conditions · Recent changes*, with a mutually-exclusive "None"). Gloss jargon (MTC/MEN 2) inline.
- **Rewrite (conditions subhead):** keep "A few quick safety questions" + the "never rules you out" reassurance; add a small "What's this?" expander on "medullary thyroid cancer (MTC) or MEN 2."

2.3 One result moment, not two · [EXP] 415:9327/9328 + 420:11763/12081

- **What:** Collapse the early "likely clinically appropriate / not a candidate" screens into the **single match-result** screen. Don't tell the user twice whether they qualify.
- **Why current fails:** Two separate "are you a candidate?" verdicts create a second evaluation moment and a confusing double-reveal; the interim verdict adds anxiety with no payoff.
- **Rationale:** Principle 5 (act decisively, don't second-guess); one clear outcome per concept. Fewer screens = less drop.

2.4 Show one provider, but with a visible "See other providers" and a motivational frame · [EXP+COPY] 415:10762

- **What:** Keep the single strong match, but add a clearly visible "See other providers" path and reframe the match around what the provider *does for you*.
- **Why current fails:** Single-option matching reads as "assigned, not chosen" for a senior who values choosing their doctor (audit Step 8) — a plausible driver of the post-direct-booking schedule-rate drop.

- **Rationale:** Audit Tier 2 ("show 2–3 providers, or 1 with a clearly visible 'see other providers'"); provider value is *motivational accountability*, lead with the outcome not the credential ([findings #27](#); [principles #6](#)). AARP: telehealth "best with a known provider."
- **Rewrite:** match line -> "Dr. Desai will get to know your goals and keep you on track — adjusting your plan and medication as you go." + secondary "See other providers."

TIER 3 — GLP-1-specific trust (convert the GLP-1-curious senior)

3.1 Address form factor (pill vs pen) on the result screen · [COPY] [420:11763](#)

- **What:** Name the medication *options* — including a daily pill — when the result says a GLP-1 may be right.
- **Why current fails:** Form factor is the **first** thing this audience decides on (pill > pen >> injection >> compounded), yet the flow never mentions it. A pill-preferring senior (Jennifer) can't tell if Bold offers anything but injections.
- **Rationale:** [findings #1–2](#) (form factor overrides everything; "Pills easy"); [clinical-protocol §6](#) now includes oral options (Foundayo®, oral Wegovy®). **Honesty guardrail:** lead with pen/auto-injector for the injectable mainstays; don't imply oral = equal efficacy or guaranteed \$50-covered until Bridge coverage is confirmed (§6, §15c #4).
- **Rewrite:** "If a GLP-1 is right for you, your provider will go over your options together — a daily pill or a weekly pen — and choose what fits you best."

3.2 Normalize side effects as *managed* · [COPY] result / confirmation

- **What:** Add one calm line about side effects + provider management.
- **Why current fails:** Side effects are one of the **two top fears** (with cost); silence leaves the user's nausea fear (and Jennifer's lived churn) unaddressed, so the GLP-1-curious bounce.
- **Rationale:** [findings #3](#); [clinical-protocol §10](#) (nausea 30–50%) — surface frequency *with the management plan* so it reads managed, not terrifying.
- **Rewrite:** "Some people feel queasy when starting — that's common and usually eases. Your provider starts low, goes slow, and adjusts your dose to keep you comfortable."

3.3 Lead the "what makes Bold different" trust line · [COPY] landing / result

- **What:** Add the FDA-only / no-compounded / no-markup line as a trust beat.
- **Why current fails:** The flow doesn't differentiate Bold from the "wild west" of weight-loss meds — a live anxiety for this audience.
- **Rationale:** [glp1-messaging](#) pillar 4; reinforces the compounded-vial rejection ([findings #2](#), [principles #4](#)).
- **Rewrite:** "Only FDA-approved medications — no compounded drugs, no markup."

TIER 4 — Copy-clarity bugs & honest expectation-setting

4.1 Fix the concrete copy bugs · [COPY]

- `{weight management.}` unrendered template token on [415:10762](#) -> "weight management" (and move the period outside).
- "Schedule appointment" typo on [415:10762](#) -> "Schedule appointment" (or "Confirm my visit").
- "\$50 month copay" on confirmation [415:14676](#) -> "\$50 monthly copay."
- **Inconsistent dates** — schedule shows "Jan 12–14," confirmation shows "Monday, November 3." Use one consistent placeholder data set.

- "By tapping 'Agree', you accept the terms in the documents above." appears as microcopy on multiple coverage/result screens. Keep one plain consent line at the *single* real commit point; make every button say its **action** ("Check my coverage," "Continue," "Confirm my visit"), never "Agree."
- *Why these matter*: unrendered tokens and misspellings on a high-stakes medical flow read as "this site is sloppy/unsafe" to a cautious senior — the opposite of the trust the page needs.

4.2 Plain-language / reading-level pass · [COPY] (all screens)

- 6th–8th-grade reading level; gloss any jargon on first use (MTC/MEN 2, "in-network," "copay"). Senior playbook: 10th–12th-grade copy depresses healthcare conversion ~55%.

4.3 Set Bridge expectations honestly (commitment & retention, not raw CVR) · [COPY] result

420:11763

- **What**: Soften "We submit to Medicare to **get your GLP-1 covered**" into an honest, reassuring expectation.
- **Why current fails**: It implies instant/guaranteed coverage. The CMS Bridge reality is a **first claim that's denied by design**, then a 24–72h prior-auth and a brief approval window, plus a **one-month lifestyle-first** prerequisite before any Rx. A member who later hears "claim denied" will read it as rejection and churn.
- **Rationale**: [clinical-protocol](#) §15e #3 (denial-by-design; onboarding copy must pre-empt it) and §1 (lifestyle-first window).
- **Rewrite**: "If a GLP-1 is right for you, your provider starts the Medicare coverage step at your visit. Approval usually takes a few days — your care team handles the paperwork and confirms your exact cost before anything is prescribed."

Already built in `glp1_funnel` — port these into the Figma flow

(Quick wins; the team has solved them once, just not in the board.)

1. Warm intro screen + "No answer rules you out" (T1.4).
2. Labeled "Step X of 6" progress + persistent help phone (T2.1).
3. Comorbidities/safety split with warm framing + mutually-exclusive "None" (T1.4, T2.2).
4. \$0 / \$50 cost **cards** with "covered by Medicare" (T1.3).
5. Optional chief-complaint with **voice input + real Skip** (T2.2).
6. "Switch to phone call" (upgrade to equal/default per T1.5).

A/B test priority (first 30 days, ranked by expected CVR / show-rate lift)

1. **T1.5 Phone equal/default + reframed visit** on the schedule screen -> no-show (down) (single biggest lever).
2. **T1.2 Eligibility consent rewrite** (remove "Verified and its vendors") -> eligibility-step completion (up) (scariest drop point).
3. **T1.3 Personalized \$0 + "78% pay \$0"** vs the "\$0–\$25" range -> covered -> intake continuation (up).
4. **T1.1 Hero rewrite + "See how it works" + phone** -> landing scroll/start rate (up).
5. **T1.4 Warm intro + softened clinical screens** -> intake completion (up).
6. **T2.2 Question consolidation** (merge duplicate screens, group conditions, chips over open-text) -> mid-funnel completion (up).

Benchmark triggers (audit): if eligibility-step drop-off > 30%, move to a phone-concierge default; if overall completion < 45%, audit field count + reading level first.

Research pushback (flagged per the weight-management-research + senior-onboarding skills)

- **Hero uses a generic line, not the validated differentiator** — "Doctor-supervised weight loss" should be "Lose weight, stay strong" / "strength, not restriction" ([findings #25](#), [principles #13](#)).
- **Cost is shown as a vague range** ("\$0–\$25," "\$50 month copay") instead of the validated "78% pay \$0" + anchored \$50/mo ([principles #11](#), [findings #19/#21](#)).
- **The flow ignores the two biggest GLP-1 decision drivers** — form factor (pill) and side effects ([findings #1–3](#)). Add T3.1/T3.2.
- **"We submit to Medicare to get your GLP-1 covered" overpromises** vs the Bridge denial-by-design + lifestyle-first reality ([clinical-protocol §15e](#), §1).
- **Self-report comorbidity checkboxes won't satisfy a Bridge PA** (needs BP values + ≥ 2 meds, A1c, eGFR — [clinical-protocol §15c #3](#)). The "you're covered" verdict risks overpromising eligibility; confirm where the funnel hands off to evidence capture.

Open product questions (not mine to decide — confirm before building)

- **Non-Bridge "lifestyle" result framing** ([clinical-protocol Tensions §5](#)): does Bold enroll non-Bridge members into a lifestyle-only program with PCP coordination, or route them out? Current copy is soft on "we won't be your GLP-1 prescriber."
- **Already-on-a-GLP-1 cohort** ([open-questions](#)): needs an intake variant capturing initiation-date BMI — not handled in this flow.
- **Should "Check my coverage" be an interactive widget** vs a CTA ([open-questions](#))?

Verification (how to validate before/after shipping)

- **Moderated usability test, 5–6 adults 65+** (mix of GLP-1-naïve and previously-churned, like Jennifer). Tasks: complete enrollment to a booked visit. Watch the eligibility-consent screen, the clinical-exclusion screens, and the schedule screen specifically. Success = completes without asking "is this a scam / am I being charged / which cost is me?"
- **Instrument per-step drop-off** (the funnel has no progress data today): landing scroll %, eligibility completion, covered -> intake continuation, schedule rate, show rate.
- **A/B test in the priority order above**, copy + layout changes shipped together (copy-only tests under-read; audit caveat).
- **Read-back check** at the confirmation screen: the member can state *who* they're seeing, *when*, *by phone or video*, and *what it costs* — the commitment test that predicts show-rate.